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Embracing slippery slope on physician-assisted suicide and euthanasia could have significant unintended consequences

Zeljka Buturovic

ABSTRACT

In a recent article Joshua James Hatherley argues that, if physician-assisted suicide (PAS) is morally permissible for patients suffering from somatic illnesses, it should be permissible for psychiatric patients as well. He argues that psychiatric disorders do not necessarily impair decision-making ability, that they are not necessarily treatable and that legalising PAS for psychiatric patients would not diminish research and therapeutic interest in psychiatric treatments or impair their recovery through loss of hope. However, by erasing distinction between somatic and psychiatric disorders on those grounds, he also erases distinction between healthy adults and patients (whether somatic or psychiatric) essentially implying that PAS should be available to all, for all reasons or, ultimately no reason. Furthermore, as psychiatric patients are much more likely to be a source of usable organs for transplantation, their broad inclusion would strengthen the link between PAS/euthanasia and organ donation, potentially undermining both as well as diminishing already declining general trust in medical authorities and professionals and public health authorities and activists.

In his article 'Is the exclusion of psychiatric patients from access to physician-assisted suicide discriminatory'[1] Joshua James Hatherley argues that, if physician-assisted suicide (PAS) is morally permissible for patients suffering from somatic illnesses, it should also be permissible for psychiatric patients. Hatherley argues that there are three common reasons why PAS for psychiatric patients is usually resisted: that these patients have, in comparison to somatic patients, uniquely impaired decision-making capacity; that their conditions are comparatively more treatable and that institutionalisation of PAS for psychiatric patients poses unacceptable risks by diminishing research and therapeutic interest in psychiatric treatments as well as potentially diminishing effectiveness of psychiatric treatment through loss of hope endangered by the very existence of PAS as an option.

While Hatherley's argument is curiously limited to PAS, it is easily extendable to euthanasia and thus applicable to a broader category of medical assistance in dying (MAID). The crux of Hatherley's argument relies on the view that psychiatric patients are not fundamentally different from somatic patients in respect to reasons listed. Thus, Hatherley argues that psychiatric patients' decision-making is not distinctly compromised, that their conditions are not always treatable and that provision of PAS would not significantly diminish interest in research and provision of psychiatric treatments. Hatherley also argues that availability of PAS introduces a similar element of hopelessness to both psychiatric and somatic patients and therefore, even if it does increase the risk of premature (physician-assisted) death, it does so similarly in both cases.

This type of argument is often raised by opponents of euthanasia who forever warn about the slippery slope from a quadriplegic patient with cancer in unbearable pain to a healthy individual suffering from nothing more than ennui and, increasingly, by those such as German Federal Constitutional Court,[2] who see MAID as a special case of a right to a self-determined death. Unlike them, Hatherley only makes the first step on the slippery slope. However, the very point of the slippery slope is that stopping at any particular point amounts to inconsistency.

Consider, in the light of Hatherley's three criteria, a case of a healthy adult who has suffered the unspeakable tragedy of losing a child. Suppose this person considers suicide and demands her physician provides her with the means that will make her death certain and physically comfortable. Her loss is irreversible, she believes that unbearable suffering she is experiencing will be with her forever and nobody can guarantee that this will not be the case. Meanwhile, her decision-making capacities are as good as anyone's while it's unclear that availability of MAID would affect her hopelessness. Why should this individual not be offered PAS? By all measures Hatherley contemplates, this person is at least as justified or more justified in demanding PAS.

But the argument does not stop here either, for various other losses can feel similarly devastating even if they do not necessarily appear objectively comparable. A loss of a spouse, a parent, a sibling, a friend, a mentor or a beloved pet can cause unbearable suffering for which, too, there is no guarantee of ever subsiding. Even if the pain does slowly fade over time and the person agrees that this will be the case, the rate of recovery can be so slow that someone of an advanced age or even middle age might realistically conclude that they will spend the rest of their lives waking up with the unbearable thought that their loved ones are gone.

Then, there are other common triggers of suicide—a loss of a job, public humiliation, feeling of guilt for unleashing pain on others (eg, causing death through traffic accident or professional incompetence). All these situations and many others, increasingly less 'objectively' deserving of suicide (from disability to unrequited love to a closing of a favourite restaurant) easily—in fact usually more easily than the case of psychiatric patients—justify provision of PAS as understood through Hatherley's criteria.

Majority of somatic patients, psychiatric patients, as well as ordinary adults in all kinds of circumstances never as much as seriously reflect on the possibility of ending their lives—to the contrary, many make extraordinary efforts to prolong them. However, a fraction of somatic and psychiatric patients as well as adults who, for whatever reason (a reason that the rest of us might or might not relate to) subjectively feel that their lives are unbearable and express a desire for MAID all meet Hatherley's criteria. This stems from the fact that, at the end of the day, his criteria, apart from a desire to end one's life and a lack of guarantee that such desire will eventually abate do not require anything but a decision-making capacity that is not significantly compromised and a passable argument that availability of MAID does not increase hopelessness above the baseline set by terminal somatic patients.

However, there are multiple reasons not addressed by Hatherley that one needs to keep in mind when considering extending PAS to psychiatric patients (to say nothing of other groups).

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TRUST IN MEDICAL AUTHORITIES AND PROFESSIONALS IS DECLINING

Public trust in the medical profession is in decline.[3] So is compliance with public authorities' recommendations. For example, vaccination rates in several countries have decreased causing measles outbreaks, a pattern that tends to be correlated with distrust in health staff.[4] In the USA, there is a significant lack of trust in media, government and, increasingly, colleges and universities—institutions that the medical profession heavily relies on as an intermediary between medical knowledge and the general population.

Other health-related industries fare no better. Pharmaceutical companies are reviled around the world for their perceived profit-making priorities and even in the USA, close to a half of Americans have a 'negative' or 'very negative' view of pharmaceutical companies.[5] While health insurance companies in the USA have a better rating overall, they are still unpopular enough to routinely be portrayed as villains during political campaigns.

Psychiatry, in particular, is held in low esteem. In the UK, psychiatry has been described as 'being on the ropes'[6] and psychiatrists as an 'endangered species'.[7] In the USA, psychiatrists have a reputation that is closer to that of bankers than medical doctors and dentists.[8]

It is doubtful that freely dispensing or even casually contemplating offering suicide pills to their patients will help reverse these trends. To the contrary, in a world where providing such pills has a potential to save billions of dollars in therapy, one can expect conflict of interest between psychiatrists, pharmaceutical companies and insurance companies to be often suspected. A declining trust in expertise and institutions puts psychiatry in particular, and medical profession and related health fields more broadly, at the point where prudence in entertaining radical ideas is advised.

THE LINK BETWEEN EUTHANASIA AND TRANSPLANTATION WOULD BE SIGNIFICANTLY STRONGER IF PSYCHIATRIC PATIENTS WERE OFFERED PAS

Finally, there exists a rising awareness that euthanasia (and relatedly, PAS) could be connected to transplantation.[9 10] Given organ shortage, logic of Pareto efficiency suggests that organs of patients who are euthanised (or use PAS) should not be wasted if those patients explicitly demand that their organs be donated. Currently there have been about several dozen cases of organ donation in euthanised patients.[11] Concerns with the quality of organs have led to proposals that dead-donor-rule be abandoned and that the euthanasia patient be sedated while their organs are removed and then eventually euthanised by removal of their heart (patients waiting for organs would simultaneously be prepared for transplantation nearby).

While efforts are made to administratively separate euthanasia requests from the process of organ donation, it is very difficult to separate them logistically and psychologically as well in the court of public opinion. Psychiatric patients are at a particular risk because their organs are likely to be, on average, in a significantly better shape than those of patients suffering from cancer (who comprise a vast majority of euthanised patients). Awareness that their death could be helpful to others would therefore play a bigger role in psychiatric than in somatic patients. Furthermore, participation of psychiatric patients in euthanasia/PAS would many times increase proportion of organs used from euthanised patients, making the connection between euthanasia/PAS and organ donation much more visible in the public eye.

As the enduring appeal of movies like 'Coma' illustrates, a fear that one might be abducted or killed for his organs has been alive in the public mind for a long time. However, social media allows disinformation and conspiracies to blossom among like-minded people on an unprecedented scale. A highly visible connection between PAS (or euthanasia) and organ harvesting could turn the public against transplantation,[12] to say nothing of euthanasia/PAS.

CONCLUSION

While it is absolutely crucial that ideas such as extending PAS/euthanasia to psychiatric patients are freely discussed on the pages of bioethics journals, serious or even casual considerations of these possibilities by medical authorities and practitioners could feed into paranoia and distrust of institutions and have costs that significantly override any potential benefits.

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